

Three ways to get on the board

Two close in days, one is the standing gate. Point by point: what each is, KOR's angle, and exactly what you do — with owners.

AT A GLANCE

The three, ranked by clock

#	OPPORTUNITY	CLOSES	KOR'S ROLE	REALISTIC?
1	Interior Health — RIH "ERCP" A&E Services (Kamloops)	Jul 13, 2 PM	Structural sub to an architect prime — <i>if</i> structural is even required	Uncertain — pull the BC Bid package first; three unknowns to resolve today
2	CFHA Comox & Esquimalt housing (DND/DCC)	Jul 14, 2 PM ET	Structural sub on a design-build prime's bid	Yes — it's relationship + package, not a KOR submission
3	LMFM RFPQ26-PE — health structural roster	Rolling (Sep 30)	KOR submits directly to the roster	Yes — ~90% assembled; the durable gate to all Lower Mainland health work

The three share the same raw material — the four engineer resumes and the seniors/care references — so the gather-work overlaps. Do it once, use it three times.

Pull the package before you spend an hour on it

What it is — honestly, we don't fully know yet, and that's the finding. BC Bid process 230528, "Architectural & Engineering Services," closes **July 13, 2 PM**. It sits inside Interior Health's RIH capital program (the \$490M Patient Care Tower / ED expansion — the final trauma & resuscitation zone is the not-yet-designed piece, targeted spring 2027). But three things are unverified, and each could kill the effort:

THREE UNKNOWNNS — RESOLVE BEFORE INVESTING ANY BID TIME

- **"ERCP" can't be verified anywhere.** No public Interior Health use of that acronym as a project term exists (the only "ERCP" they publish is an unrelated GI procedure). We don't actually know the scope until the document is open.
- **Structural may not even be in scope.** The RFP's own discipline tags list Architectural / Electrical / Mechanical / Hospital construction — *no Structural tag*. Confirm structural is required before anything else.
- **KOR has zero hospital references.** Real regional presence (the Kelowna office under Conor Murtagh, Kamloops built work) — but no healthcare projects anywhere in the portfolio. If the RFP makes acute-care experience a mandatory pass/fail for each named sub, that's a hard disqualifier, not a scoring ding.

THE REALISTIC READ

Bidding as prime in two days is not achievable — an acute-care A&E procurement needs an AIBC architect leading a full team, references, and a sealed fee schedule. The *only* credible path is as a **named structural sub to an architect prime already assembling a team** — and only if one needs a structural sub today. So this is a phone play, not a proposal.

DO THIS - TODAY, IN ORDER

- **Log into BC Bid, open 230528, download the package** — RFP/Terms of Reference, the consultant-discipline schedule, the evaluation matrix, and the addenda + plan-takers list. *you – only you have the login first thing*
- That one download answers all three unknowns: what ERCP is, whether structural is required, and whether subs need their own hospital references. **10 minutes**
- **If structural is in and subs don't need independent hospital refs** — call the BC healthcare architecture primes on the plan-takers list and ask who still needs a structural sub. **same morning**
- **If it's a standing offer** (the title says "Service Delivery" — it may be an ongoing on-call roster, not one project) — that's the more valuable outcome and worth the push even without a specific building. *confirm from the doc*
- **If structural isn't required, or subs need hospital refs KOR doesn't have** — drop it and put the hours into CFHA and LMFM, where KOR actually fits. **don't sink time into a pass/fail we'd fail**

Bottom line: this is the one of the three where the honest move is *investigate, then decide* — not assume it's a win. Ten minutes with the real document tells you whether to chase it or let it go.

Get on a prime's team before Jul 14

What it is. DND/CFHA military housing — 212 units Comox + 208 units Esquimalt, ~\$184–232M combined, procured as a Modified Design-Build through Defence Construction Canada. MERX PR26RCIP_Pacific, **closes July 14, 2 PM ET** (amended from June 30). ~83–209 plan holders.

THE KEY POINT

KOR does not submit this — the design-build prime does. Your job is to be named as their structural sub *before* they bid. So the real deadline is now, not the 14th, and the deliverable is a call + a one-page package, not a proposal.

DO THIS

- Call the design-build primes likely bidding — [PCL](#), [Bird](#), [EllisDon](#), [Pomerleau](#), [Chandos-Bird](#) — and say "KOR is your structural sub on PR26RCIP_Pacific." **before Jul 14**
- Confirm the actual bidders on MERX (Plan Holders tab, needs your login) so you're not cold-calling — [MERX login](#)
- Hand each prime the one-page KOR structural package: capability blurb + 4–5 multifamily/seniors references + the 4 resumes + \$5M PL + WorkSafeBC. **same call**
- Mention clearance-readiness: KOR isn't cleared yet, but primes can't require it to name you, and the AFR sponsorship rides their bid — [ref the qualification-path brief](#)

DCC contact for the notice: Andrew Webster, 343-553-9546, andrew.webster@dcc-cdc.gc.ca. **KOR's fit:** exactly the multifamily wood-frame/concrete work KOR does daily — the references (Dania, seniors campuses, multifamily) are directly on point.

The Lower Mainland health roster — finish and submit

What it is. The Pre-Qualified Professional Engineers roster shared by **VCH / Fraser Health / PHSA** (Structural discipline). Getting on it is the gate to all Lower Mainland health work. Submit **Non-Acute only**, 5 replacement references, on Bonfire opportunity 101915 (two PDFs: Application = Appendices A+C+D+E+F, and Resumes). **Deadline Sep 30 but rolling — submit early.** ~90% done.

DECISIONS ONLY YOU / THE PRINCIPALS CAN MAKE

- Approve the **5 replacement references**: Guru Nanak, Belmont Village La Jolla, Dania, Weave, Harrison Healthcare
- **Guru Nanak** — will it be complete before Sep 30? If not, it's out (swap in another seniors-care ref)
- **Litigation answer** (Bryson/BMZ) — legal confirms "No / No / No"
- **Contracting entity** — which entity holds EGBC Permit 1000378 and signs (Daler/finance)

TO GATHER / PRODUCE (PMS + YOU)

- **Resumes**: Markulin, DesRoches, Murtagh, Beirne — skeletons are drafted (attached), PMs fill years / EGBC # / experience
- **Missing project fields**: a few budgets, contract types, and direct reference contacts for PICS and Belmont (main lines are on file)
- **Sign** Appendix D (Relationship Disclosure) + Appendix E (Funding Disclosure)
- **You**: fill Appendices A/C/D/E, combine the two PDFs, submit on Bonfire 101915

Everything else is done: firm legal name, EGBC permit, signatory (Markulin), bank ref (Scotiabank), \$5M PL (BFL), diversity narrative (EGBC Accredited Employer), reference phone numbers. Full detail: KOR-RFPQ26-PE-Project-Reference-Dossier.

Do these once, they serve all three

- **The 4 resumes** (Markulin, DesRoches, Murtagh, Beirne) — needed for LMFM, the CFHA sub package, and RIH. Skeletons drafted; PMs finish.
- **The seniors/care reference set** — Dania, Guru Nanak, Belmont, Weave, Fraserview, plus the deeper Deltek list — feeds all three.
- **A one-page KOR structural capability sheet** — reusable for CFHA primes and RIH teaming. (Say the word and I'll draft it.)

KOR Structural — Confidential / Internal · Prepared overnight 2026-07-11 for Ian · Sources: BC Bid 230528 (RIH), MERX PR26RCIP_Pacific (CFHA), Bonfire 101915 + KOR-RFPQ26-PE tracker (LMFM), KOR Deltek portfolio. RIH scope researched against Interior Health / news.gov.bc.ca.